



Nursing Practice Guideline

Radiological Examination and Neuro Interventional Procedures

Reference #: NPG26

Scope

Site	Service/Department/Unit	Disciplines
SCGH	Hospital Wide	Nursing

Introduction

This guideline outlines the procedures and responsibilities of individuals and the health service organisation in relation to Radiological examination and Neuro interventional procedures. It ensures the delivery of a uniform, integrated multidisciplinary approach with management that is consistent with best practice.

Risk Statement

Non-compliance with this guideline will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☐
Breach SCGH Mission & Values	☒	Staff Safety	☒
Other:	☐		

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These guidelines are intended to guide clinical decision making for clinical staff at North Metropolitan Health Service. They are not strict protocols and do not replace clinical judgement. These guidelines should never be relied on as a substitute for proper assessment of the particular circumstances of each case, and the needs of each patient. If staff decide, in exercising their clinical judgment, not to follow the guideline, it is recommended that the reasons are clearly documented in the patient's health care record.

1. General Instructions

1.1 Medical Requirements

All radiological and interventional radiological procedures are performed within Radiology by a Radiologist, Vascular Surgeon, or Renal Physician. All patients for interventional procedures should be assessed for confusion, and the ability to lay flat / "still" for a period of time. If the patient is unable to lay flat / "still" for a period of time, then the referring medical officer should arrange an anaesthetic assessment prior to a booking being accepted.

Consent must be obtained by the Proceduralist, if indicated as a requirement in the specific procedural information table^{1,2}.

Due to the high risk of contrast induced nephrotoxicity **ALL** patients requiring contrast media must have a recent creatinine / eGFR.¹ **If eGFR < 30 refer to Vascular Department Guideline Pre-Hydration for Patients with Renal Impairment Receiving Radiological Contrast Agents**

Patients attending Magnetic Resonance Imaging (MRI) must have a completed MRI safety checklist; and Inpatient Letter checking for pacemakers, cochlear implants, and aneurysm clips.

All patients with allergies to iodine must be documented on the Radiology request form, staff to contact the relevant Radiology area to discuss the management of the patient.^{1,3}

1.2 Safety Information

Patient identification is confirmed prior to the commencement of any radiological intervention.

Patients should always be asked if there is any possibility of pregnancy. If unsure, a pregnancy test to be performed prior to coming to Radiology.

Patients requiring interventional and/ or procedural treatment are required to have a nurse escort to and from the department to provide appropriate ISOBAR Clinical Handover

Non-invasive procedures do not require an escort or patient handover unless clinically indicated between 0830-2230 Monday – Sunday.

Note: After hours (between 2230- 0830) a nurse escort is required for any patient of concern or in escalation criteria, have any IV additives, or drains insitu, are on high flow oxygen and may be required to remain with the patient for the duration of the procedure.

1.3 Nursing Management

- Diabetic patients – Check medication (i.e., insulin, metformin) and a recent blood sugar level must be documented on Radiology Checklist. If patients receive IV contrast and are on metformin, withhold for 48hrs post procedure (monitor BSLs minimum of QID).
- Medical / nursing notes and relevant x-rays (if not available on PACS) must accompany the patient to the Radiology Department.
- Patients should be showered prior to an interventional procedure.
- *S aureus* decolonisation is required for patients scheduled for insertion of a select group of vascular access and catheter devices, specifically:

- Totally Implantable Venous Access Devices (eg Infusaport, Port-a-cath, PAS-Port)
- Non-tunnelled large bore catheter (Vas-Cath)
- Tunnelled (subcutaneous) large bore catheters (e.g., Hickman, Broviac, Groshong)
- Peritoneal dialysis (PD) catheter insertion

NB. decolonisation is NOT currently recommended for routine insertion of short to medium term catheters e.g., PICCs, temporary percutaneous CVCs.

Please refer to the Mupirocin Nasal Ointment for Decolonisation Clinical Practice Guideline for detailed information.

<https://healthpoint.hdwa.health.wa.gov.au/policies/Policies/NMAHS/SCGH/SCGOPHCG.CPG.MupirocinNasalOintmentforDecolonisation.pdf>

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- Any clinical information enquiries for patients attending the following areas please call appropriate extension:

SCGH:

- | | |
|-----------------------------|--------------------|
| ○ General X-Ray | ext. 72693 / 71061 |
| ○ Ultrasound | ext. 74802 |
| ○ Barium Studies/IVP | ext. 74835 |
| ○ CT Scan Ground Floor | ext. 71160 |
| ○ CT Coordinator | ext. 73030 |
| ○ Interventional Procedures | ext. 73479 |
| ○ MRI Nurse Coordinator | ext. 77455 |

OPH

Radiology

ext 78150

1.4 Procedural Information

A Procedural Checklist must be completed for each patient having an invasive procedure.

*All medications should be administered with minimal fluid, unless otherwise stated.

Patients requiring peripheral vascular access should have as a minimum a 20g cannula inserted.

Interpreters are to be booked for non-English speaking patients; interpreters are to attend procedure / scan with patient.

Please refer to NPG 55 'Preoperative Care' for Sip til Send information

2. Computer Tomography (CT) Scan

PROCEDURE	WRITTEN CONSENT	CLIP	FASTING	PREPARATION	DOCUMENTATION (type of checklist required)	OBS PRE	OBS POST	NOTE	PROC TIME (APPROX)
CT Head	No	No	No	Hospital gown IV cannula 20 gauge if contrast required	Contrast Consent done in CT	Core Physiological Observations	N/A		5 mins
CT Chest	No	No	No	Hospital gown IV Cannula 20 gauge	Contrast Consent done in CT	Core Physiological Observations	N/A		10-15 mins
CT Coronary	Yes	No	No	Hospital gown IV cannula 18 gauge in right cubital fossa. No Caffeine for 24 hours	Medication chart Contrast Consent done in CT	Core Physiological Observations	As per Radiologists Instructions		1 -2 hrs
CT Abdomen, Pelvis	No	No	No	Hospital gown IV cannula 20 gauge <u>Contrast CT</u> 1) Positive Oral Contrast as per Radiologist Omnipaque 300 x 20mls in 1 litre of H ₂ O 2) Non-Positive Contrast 1 Litre of water over 1 hour which can be consumed on the ward <u>Non-Contrast CT</u> No prep or IV required	Contrast Consent done in CT	Core Physiological Observations	N/A	Studies of large bowel will take longer if positive oral contrast is needed as contrast transit times are longer	10 - 15 mins

Radiological examination and Neuro interventional procedures

PROCEDURE	WRITTEN CONSENT	CLIP	FASTING	PREPARATION	DOCUMENTATION (type of checklist required)	OBS PRE	OBS POST	NOTE	PROC TIME (APPROX)
CT Cholangiogram	No	No	Fasting solids and fluids 4hrs	Hospital gown IV cannula 20g If biliary drain in situ clamp after escorting patient to CT prior to giving Biliscopin	Contrast Consent done in CT	Core Physiological Observations	N/A	LFTs within one month	30-minute infusion time 10-minute scan time
CT Renal Colic	No	No	No	Hospital gown		Core Physiological Observations	N/A		5 mins
CT Renal Triphasic	No	N/A	No	Hospital Gown IV cannula 20g cubital fossa	Contrast Consent done in CT	Core Physiological Observations	N/A		30 mins
CT Facet Joint Injection	Yes	N/A	No	Hospital Gown	NIISwa Neurovascular Laboratory checklist MR730.6	Pain Score Core Physiological Observations Full Neurological Observations	Pain Score Full Neurological Observations	Patient to Lie prone for procedure – notify staff if this will be a problem	30 - 45 mins
CT Nerve Root Sleeve Injection	Yes	N/A	No	Hospital Gown	NIISwa Neurovascular Laboratory checklist MR730.6	Core Physiological Observations + Full Neurological Observations	Full Neurological Observations	Patient to lie prone during procedure – notify staff if this will be a problem	30 -45 mins
CT Thoracic Dissection	No	N/A	No	IV Cannula 18g in right cubital fossa	Contrast Consent done in CT	N/A	N/A		10-15 mins

Radiological examination and Neuro interventional procedures

PROCEDURE	WRITTEN CONSENT	CLIP	FASTING	PREPARATION	DOCUMENTATION (type of checklist required)	OBS PRE	OBS POST	NOTE	PROC TIME (APPROX)
Lung Biopsy	Yes	No	6hrs solids Sip till send clear fluids	Hospital gown IV cannula 20g FBC, U&E, coags <i>Pathology form for sample (Completed in Radiology)</i>	Procedural Checklist	Core Physiological Observations	Assess BP, pulse, respiration rate 15/60 for 2 hr 30/60 for 2 hr Hourly for 2hrs Pain Score	Post procedure: Lie on Biopsy side until 1 st chest x-ray reviewed CXR as per Radiologists post-op instructions O ₂ may be required as per instructions.	60 mins
Other Biopsies and Drainages	Yes	No	6hrs solids Sip til send clear fluids	Hospital gown IV cannula 20g FBC, U&E, coags <i>Pathology form for sample (Completed in Radiology)</i>	Procedural Checklist	Core Physiological Observations	Assess BP, pulse, respiration rate 15/60 for 2 hr 30/60 for 2 hr Hourly for 2hrs Pain Score	Post procedure: as per Radiologists post-op instructions	60 mins
Cryoablation	Yes		6hrs solids Sip till send clear fluids	Hospital gown & disposable pants FBC, U&E, coags, LFT's IV Cannula	Procedural Checklist	Core Physiological Observations	15/60 for 2 hours 30/60 for 2 hours Hourly for 2 hours Observe site for bleeding	As per post procedure instruction	2-4 hours

Radiological examination and Neuro interventional procedures

PROCEDURE	WRITTEN CONSENT	CLIP	FASTING	PREPARATION	DOCUMENTATION (type of checklist required)	OBS PRE	OBS POST	NOTE	PROC TIME (APPROX)
Virtual Colonography (Large bowel study)	Yes	No	Clear fluids 24 hours prior to procedure Fast clear fluids from 2400	Hospital gown Instructions Provided by CT Staff	Procedural Checklist	Core Physiological Observations	N/A	20mls Ultravist in 1L water, give 50mls only one hour prior to scan given in Radiology	1 hour prep time Scan time 30 mins
CT Enterography (Small Bowel Study)	Yes	No	Fast solids from midnight Clear fluids okay	Hospital gown Instructions provided by CT staff	Procedural Checklist	Core Physiological Observations	N/A	Picoprep 8pm the night before	30 mins
CT Swallow – query leaks	No	No	No	Hospital gown 20g IV Cannula	Procedural Checklist	N/A	N/A	10% solution 50mls Ultravist in 500mls H ₂ O given in Radiology	

3. Fluoroscopy

PROCEDURE	WRITTEN CONSENT Yes or No	CLIP	FASTING	PREPARATION	DOCUMENTATION (type of checklist required)	OBS PRE	OBS POST	NOTE	PROC TIME (APPROX)
Barium Swallow and/or Meal	No	No	Fast solids 6hrs Fast liquid 2hrs Avoid smoking and chewing gum.6hrs	Hospital gown	Procedural Checklist	Core Physiological Observations	N/A	Encourage fluids post procedure unless contraindicated Administer aperient as prescribed	15-30 mins
Speech fluoroscopy	No	No	No	Nil	Procedural Checklist	-	-	N/A	5-10 mins
Barium Enema (Single Contrast)	No	No	Fast solids 6hrs Fast liquids 2hrs	Hospital gown	Procedural Checklist	Core Physiological Observations	N/A	Additional (spare) ostomy bag to be sent if patient has an Ileostomy or colostomy	30 - 45 mins
T-Tube Choledocho- Cholangiogram / Pegogram/ RIGS	No	No	Not applicable	Hospital gown	v	Core Physiological Observations	N/A		5 - 10 mins
Micturating Cystogram	No	No	Not applicable	Hospital gown Require IDC pre- procedure	Procedural Checklist	Core Physiological Observations	N/A	Outpatients require admission via DPU	30 – 45 mins

Radiological examination and Neuro interventional procedures

PROCEDURE	WRIT TEN CONS ENT	CLIP	FASTING	PREPARATION	DOCUMENTATION (type of checklist required)	OBS PRE	OBS POST	NOTE	PROC TIME (APPROX)
Retrograde Pyelogram (Usually associated with an operative procedure)	Yes	No	Not applicable	Hospital gown & disposable pants	Procedural Checklist	Core Physiological Observations	As per post-operative obs	Outpatients require admission via DPU	20 – 30 mins
Insertion of NG/NJ Tube (Small Bowel Intubation)	Yes	No	Fast from midnight	Hospital Gown	Procedural Checklist	Core Physiological Observations	N/A	N/A	30 – 60 mins
Urethrogram	Yes	No	Not applicable	Hospital Gown	Procedural Checklist	Core Physiological Observations	N/A	N/A	20 – 30 mins
Fistulogram/ Sinogram	Yes	No	Not applicable	Hospital Gown	Procedural Checklist	Core Physiological Observations	N/A	N/A	20 – 30 mins

4. Interventional Radiology Procedures

PROCEDURE	WRITTEN CONSENT	CLIP	FASTING	PREPARATION	DOCUMENTATION (Type of checklist required)	OBS PRE	OBS POST	NOTE	PROC TIME APPROX
Angiogram/ Angioplasty / Stent - Femoral - Renal - Mesenteric - Arch -Thoracic	Yes	Bilateral groin	6hrs solids Sip till send clear fluids	Hospital gown & disposable pants IV cannula 20g FBC, U&E, Coagulation Profile if patient on anticoagulants , has liver disease or medically unstable If on metformin, withhold for 24hrs pre procedure (monitor BSLs QID.	Procedural Checklist	Core Physiological Observations plus full neurovascul ar observations to affected limb (ensure to mark pedal pulses)	Check puncture site for haematoma. Full neurovascul ar observations 15 /60 for 1 hour, 30/60 for 1 hours, then hourly for 2 hours. Refer to operation report for post op instructions, including mobilisation time and puncture site monitoring Encourage fluids post procedure unless patient is on a fluid restriction.	Patients having anticoagulation therapy need to be reviewed by the medical team in conjunction with the Interventional Radiologist If on metformin, withhold for 48hrs post procedure (monitor BSLs). Bloods: If no renal dysfunction – U&E 1-week post op. If renal impairment – U&E Day 1 and day 7 post-op. Other bloods as clinically indicated only If closure device is used follow post op instructions. If bleeding apply direct pressure to puncture site and notify the medical officer urgently	3 – 4 hrs minimum

Radiological examination and Neuro interventional procedures

PROCEDURE	WRITTEN CONSENT	CLIP	FASTING`	PREPARATION	DOCUMENTATION (Type of checklist required)	OBS PRE	OBS POST	NOTE	PROC TIME APPROX
Angiogram plus Thrombolytic Therapy	Yes	Bilateral groin	6hrs solids Sip till send clear fluids	Hospital gown & disposable pants IV cannula 20g. Insert IDC- females only Refer to Acute Pain Service for PCIA if required Pressure relieving mattress Will require ICU bed post procedure	Procedural Checklist	Core Physiological Observations plus full neurovascula r and neurological observation	As per department guideline #12 - Peripheral Intra-Arterial Thrombolysis Full Neurological and Neurovascul ar Observations	No I.M or SC injections. Requires ICU. If on metformin, withhold for 48hrs post procedure (monitor BSLs).	2 - 4hrs

Radiological examination and Neuro interventional procedures

PROCEDURE	WRITTEN CONSENT	CLIP	FASTING`	PREPARATION	DOCUMENTATION (Type of checklist required)	OBS PRE	OBS POST	NOTE	PROC TIME APPROX
T.I.P.S.S. (Trans Jugular Intra Hepatic Porto- Systemic Shunt)	Yes	N/A	6hrs solids Sip till send clear fluids	Hospital gown & own pants FBC, U&E, Coag Profile IV Cannula 20g	Procedural Checklist	Core Physiological Observations	As per anaesthetist. Observe neck for swelling.		2 - 4 hrs
R.I.G (Radiology Inserted Gastrostomy Tube)	Yes	N/A	6hrs solids 4hrs fluids	Hospital gown & own pants FBC, U&E, Coag Profile IV Cannula 20g	Procedural Checklist	Core Physiological Observations	As per anaesthetist. Observe neck for swelling.		2 - 4 hrs

Radiological examination and Neuro interventional procedures

PROCEDURE	WRITTEN CONSENT	CLIP	FASTING`	PREPARATION	DOCUMENTATION (type of checklist required)	OBS PRE	OBS POST	NOTE	PROC TIME APPROX
Nephrostomy Tube Insertion	Yes		6hrs solids Sip till send clear fluids	Hospital gown FBC, U&E, Coag Profile IV Cannula 20g	Procedural Checklist	Core Physiological Observations	15/60 for 2 hours 30/60 for 2 hours Hourly for 2 hours	Patient will need to lie prone for procedure – notify Interventional Radiology staff if this will be a problem.	1 -2 hrs
Ureteric Stent Insertion	Yes	N/A	6hrs solids Sip till send clear fluids	Hospital gown IV Cannula 20g FBC, U&E, Coag Profile	Procedural Checklist	Core Physiological Observations	As per post procedure instructions	Patient to lie prone for procedure – notify Interventional Radiology staff if this will be a problem.	30-60mins
Hickman Line Insertion	Yes	Complete in Radiology	6hrs solids Sip till send clear fluids	Hospital gown & own pants Chlorhexidine 4% wash Nasal Mupirocin 2% - one application FBC, U&E, Coags Profile	Procedural Checklist	Core Physiological Observations	Check for bleeding. As per post procedure instructions .	Nasal Mupirocin to be prescribed for post insertion as per CPG Mupirocin Nasal Ointment for Decolonisation	1 hr

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Infusaport Insertion	Yes	As required	6hrs solids Sip till send clear fluids	Hospital gown & own pants Pre-Procedure 2% nasal Mupirocin ointment, and 4% chlorhexidine wash as per the Mupirocin Nasal Ointment for Decolonisation Guideline. IV cannula FBC, U&E, Coag Profile	Procedural Checklist	Core Physiological Observations	Check for bleeding. As per post procedure instructions		1 hr
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Radiological examination and Neuro interventional procedures

PROCEDURE	WRITTEN CONSENT	CLIP	FASTING`	PREPARATION	DOCUMENTATION (type of checklist required)	OBS PRE	OBS POST	NOTE	PROC TIME APPROX
Endovascular aneurysm repair: - EVAR - FEVAR - TEVAR	Yes	Bilateral groin	6hrs solids Sip till send clear fluids	Hospital gown & disposable pants Graduated compression stocking (e.g. TEDS) / Flowtrons as per Vascular surgeon instructions FBC, U&E, Coag Profile and Group and Hold	Procedural Checklist	Core Physiological Observations plus full neurovascular observations to bilateral feet	Check puncture site for haematoma Core Physiological Observations plus neurovascular obs 15 /60 for 1 hour, 30/60 for 1 hour, hourly for 2 hours, and 2 hourly for 4 hours Refer to operation report for post op instructions, including mobilisation time Encourage fluids post procedure unless patient is on a fluid restriction.	Antibiotic cover as per Vascular Surgeon Bloods: If no renal dysfunction – U&E 1 week post op. If renal impairment – U&E day 1 and day 7 post-op. Other bloods as clinically indicated only	2- 3 hrs

Radiological examination and Neuro interventional procedures

PROCEDURE	WRITTEN CONSENT	CLIP	FASTING`	PREPARATION	DOCUMENTATION (type of checklist required)	OBS PRE	OBS POST	NOTE	PROC TIME APPROX
Uterine Fibroid Embolisation	Yes	No	6hrs solids Sip till send clear fluids	Hospital gown - Insert urinary indwelling catheter FBC, U&E, Coag Profile	Procedural Checklist	Core Physiological Observations Full baseline neurovascular observations	15/60 for 1 hour 30/60 for 1 hour Hourly for 1 hour as per post procedure instructions Check for bleeding rest in bed 4hrs. PCIA obs as per NPG #8 - Opioid and Ketamine Intravenous Analgesia Pain relief may include: Celebrex, Oxycodone and Panadol	Pain management as per Interventional Radiology protocol post embolisation	1-2 hrs

Radiological examination and Neuro interventional procedures

PROCEDURE	WRITTEN CONSENT	CLIP	FASTING`	PREPARATION	DOCUMENTATION (type of checklist required)	OBS PRE	OBS POST	NOTE	PROC TIME APPROX
Prostate Artery Embolisation	Yes	No	6hrs solids Sip till send clear fluids	Hospital gown -FBC, U&E, Coag Profile IV Cannula 20g	Procedural Checklist	Core Physiological Observations	15/60 for 1 hour 30/60 for 1 hour Hourly for 1 hour as per post procedure instructions Check for bleeding rest in bed 4hrs punction site observation s Check puncture site for bleeding		3 – 4 hours
Vena Cava Filter Insertion/ Removal	Yes	No	6hrs solids Sip till send clear fluids	Hospital gown IV cannula 20g FBC, U&E, Coag Profile	Procedural Checklist	Core Physiological Observations	15/60 for 1 hour 30/60 for 1 hour Check puncture site for bleeding		30mins

Radiological examination and Neuro interventional procedures

PROCEDURE	WRITTEN CONSENT	CLIP	FASTING`	PREPARATION	DOCUMENTATION (type of checklist required)	OBS PRE	OBS POST	NOTE	PROC TIME APPROX
Biliary Drainage/ Percutaneous Transhepatic Cholangiogram (PTC)	Yes	Completed in Radiology	6hrs solids Sip till send clear fluids	Hospital gown & own pants FBC, U&E, coags, LFT's IV Cannula	Procedural Checklist	Core Physiological Observations	As per Anaesthetis t. Observe site for bleeding	Manageme nt as per post procedure and Anaesthetic instructions	2 hrs

Radiological examination and Neuro interventional procedures

PROCEDURE	WRITTEN CONSENT	CLIP	FASTING`	PREPARATION	DOCUMENTATION (type of checklist required)	OBS PRE	OBS POST	NOTE	PROC TIME APPROX
TACE trans arterial Chemo- embolisation	Yes	Completed in Radiology	6hrs solids Sip till send clear fluids	Hospital gown PCA to be organised Pre Med FBC, U&E, coags, LFT'S IV Cannula 20g	Procedural Checklist	Core Physiological Observations	15/60 for 1 hour 30/30 for 1 hour Hourly for 1 hour PCIA obs as per NPG #8 - Opioid and Ketamine Intravenous Analgesia Check for bleeding Rest in bed 4hrs.	Pain management as per Interventional Radiology protocol	2hrs.

Radiological examination and Neuro interventional procedures

PROCEDURE	WRITTEN CONSENT	CLIP	FASTING`	PREPARATION	DOCUMENTATION (type of checklist required)	OBS PRE	OBS POST	NOTE	PROC TIME APPROX
Radiofrequency Ablation	Yes	No	6hrs solids Sip till send clear fluids	Hospital gown & disposable pants FBC, U&E, coags, LFT's IV Cannula	Procedural Checklist	Core Physiological Observations	15/60 for 2 hours 30/60 for 2 hours Hourly for 2 hours Observe site for bleeding	As per post procedure instruction	2-4 hours
Liver Biopsy	YES	No	6hrs solids Sip till send clear fluids	Hospital gown FBC, U&E, coags, IV Cannula 20g	Procedural Checklist	Core Physiological Observations	15/60 for 2 hours 30/30 for 2 hours Hourly for 2 hours As per post op instructions Check site for bleeding		30 – 40 mins
Cryoablation	Yes		6hrs solids Sip till send clear fluids	Hospital gown & disposable pants FBC, U&E, coags, LFT's IV Cannula	Procedural Checklist	Core Physiological Observations	15/60 for 2 hours 30/60 for 2 hours Hourly for 2 hours Observe site for bleeding	As per post procedure instruction	2-4 hours

Radiological examination and Neuro interventional procedures

PROCEDURE	WRITTEN CONSENT	CLIP	FASTING`	PREPARATION	DOCUMENTATION (type of checklist required)	OBS PRE	OBS POST	NOTE	PROC TIME APPROX
Renal Biopsy	Yes	No	6hrs solids Sip till send clear fluids	Hospital gown U&E's, Coags, FBC Urinalysis IV Cannula 20g	Procedural Checklist	Core Physiological Observations	15/60 for 2 hrs 30/30 for 2hrs Hourly for 2 hrs 4/24 for 24hrs if inpatient Rest in bed 4 hours Observe puncture site Observe for haematuria	If pt. on peritoneal dialysis – remove dwell prior to procedure Biopsy of transplanted kidney requires manual pressure 3 mins by MO, followed by sandbag pressure for 1.5 hours over biopsy site. Post biopsy native kidney: pt to remain supine position thus applying pressure over biopsy site. If day case pt. must void twice prior to discharge, with no macroscopic blood present	30 – 40 mins

PROCEDURE	WRITTEN CONSENT	CLIP	FASTING`	PREPARATION	DOCUMENTATION (type of checklist required)	OBS PRE	OBS POST	NOTE	PROC TIME APPROX
Selective Internal Radiotherapy Treatment (SIRT)	Yes	No	6hrs solids Sip till send clear fluids	Hospital gown Pre Med FBC, U&E, Coags, LFT's IV Cannula 20g Refer to Acute Pain Service for PCIA if required	Procedural Checklist	Core Physiological Observations Full neurovascular obs	15/60 for 1 hours 30/30 for 1 hour Hourly for 2 hours Observe puncture site follow Operation Report	Patient will be injected with a small amount of Radioisotope – Patients may require a single room (refer to Nuclear Physicist) No risk to pregnant staff	2 - 3 hours

5. Magnetic Resonance Imaging (MRI)

PROCEDURE	WRITTEN CONSENT	CLIP	FASTING	PREPARATION	DOCUMENTATION	OBS PRE	OBS POST	NOTE	PROC TIME APPROX
Imaging all areas of the body. Any clinical enquires call extension 77454/77455	Contrast consent	Performed in MRI suite, if required for cardiac monitoring	6hrs solids Sip til send clear fluids	Hospital gown IV cannula 22g IV sedation requires an anaesthetist MRI Safety checklist to be completed prior to transfer to unit for certain wards If being done in the MRI department and patient does not understand English an interpreter is to be arranged by the ward Medication Patches – Please notify MRI Unit if patient has a patch insitu, as it may need to be removed prior to scan in the MRI department Patients who work with metal particularly those who may have metal fragments in eyes / face Must be identified prior to procedure as may require e X-ray prior.	MRI Safety checklist MRI Inpatient Letter to be completed prior to patient attending appointment (Alert & Orientated, Micro alerts, etc.		Only required if MRI performed under general anaesthetic	Must be able to lie flat for duration of scan. Please notify MRI department if the patient is unable lie flat prior to coming down. Notify the MRI unit if the patient is known to experience anxiety or claustrophobia	15 - 60 mins

6. Ultrasound Procedures

PROCEDURE	WRITTEN CONSENT (YES or NO)	CLIP	FASTING	PREPARATION	DOCUMENTATION (type of checklist required)	OBS PRE	OBS POST	NOTE	PROC TIME (APPROX)
Abdomen	No	Only if patient is hirsute	4hrs solids Sip til send clear fluids	Hospital gown	nil	N/A	N/A		30mins
Pelvis	No	No	Drink 600mls water 1 hour prior to scan	Hospital gown Full Bladder required	nil	N/A	N/A	If patient has fluid restriction, notify radiology department	30 mins
Renal & Bladder	No	No	4hrs solids Sip til send clear fluids	Hospital Gown Full bladder may be required	nil	N/A	N/A	If patient has fluid restriction, notify radiology department	30mins
Doppler Abdominal vessels – Liver, Renal, Aorta, Iliacs	No	No	4hrs solids	Hospital gown	nil	N/A	N/A		60mins
Abdominal Paracentesis	Yes	No	4hrs solids Sip til send clear fluids	Hospital gown	Procedural Checklist	Core Physiological Observations	Refer to CPG Abdominal Paracentesis		30 – 60 mins
Deep Organ Biopsies	Yes	No	4hrs solids Sip til send clear fluids	Hospital gown INR & Platelets	Procedural Checklist	Core Physiological Observations	As per Post Op report		30 – 60 mins

Radiological examination and Neuro interventional procedures

INTERVENTIONAL	WRITTEN CONSENT (YES or NO)	CLIP	FASTING	PREPARATION	DOCUMENTATION (type of checklist required)	OBS PRE	OBS POST	NOTE	PROC TIME (APPROX)
Liver Biopsy	Yes	No	4hrs solids Sip till send clear fluids	Hospital gown IV Cannula INR & Platelets	Procedural Checklist	Core Physiological Observations	As per Post Op report		30 – 60 mins
Renal Biopsy	Yes	No	4hrs solids Sip till send clear fluids	Hospital gown IV Cannula INR & Platelets	Procedural Checklist	Core Physiological Observations	As per Post Op report		30 – 60 mins
Pleural Aspiration (Diagnostic or Therapeutic)	Yes	No	4hrs solids Sip till send clear fluids	Hospital gown IV Cannula INR & Platelets	Procedural Checklist	Core Physiological Observations	As per Post Op report		30 – 60 mins
Drainages	Yes	No	4hrs solids Sip till send clear fluids	Hospital gown IV Cannula INR & Platelets	Procedural Checklist	Core Physiological Observations	As per Post Op report		30 – 60 mins
Musculo- skeletal Biopsies	Yes	No	4hrs solids Sip til send clear fluids	Hospital gown IV Cannula INR & Platelets	Procedural Checklist				30 – 60 mins
Thyroid/ Lymph Node/ Joint Injection/ Aspiration	Yes	No	4hrs solids Sip til send clear fluids	Hospital gown	Procedural Checklist				30 – 60 mins

7. NIISwa: Neuro Interventional Procedures

Unless otherwise stated, NIISwa fasting protocols follow 'Sip til Send' allowing pre-procedural clear fluids totalling no more than 200mls per hour up until the time the department sends for the patient. Refer to NPG Pre-Operative Care for further Sip til Send details.

PROCEDURE	WRITTEN CONSENT	CLIP	FASTING	PREPARATION	DOCUMENTATION	OBS PRE	OBS POST	NOTE	Operative time
Cerebral & Spinal Angiogram +/- intervention	Yes	Yes (bilateral groins)	Minimum 6hrs solids Sip till send clear fluids	Hospital & disposable pants IV cannula 20G FBC, U&E, Coags, Group & Hold	NIISwa Neurovascular Laboratory checklist (MR730.6) Completed imaging request form	Core Physiological Observations Full neurovascular and Neurological observations	Refer to operation report for post procedure requirements. Observe Puncture site	IDC insertion, TED stocking for intervention Patient weight limits<200Kg	variable

Radiological examination and Neuro interventional procedures

Thrombin Injection	Yes	No	No	Hospital gown IV cannula 20G FBC	NIISwa Neurovascular Laboratory checklist (MR730.6) Completed imaging request form	Core Physiological Observations Full neurovascular and Neurological observations Pain assessment Haematoma assessment	Refer to operation report for post procedure requirements' RIB till medical review	Monitor patient post procedure for groin complication: pseudo- aneurysm	1hour
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Radiological examination and Neuro interventional procedures

PROCEDURE	WRITTEN CONSENT	CLIP	FASTING	PREPARATION	DOCUMENTATION	OBS PRE	OBS POST	NOTE	Operative time
Myelogram (Cervical, Thoracic, Lumbar) / Cysternogram	Yes	If needed	4hrs solids Sip til send clear fluids	Hospital gown IV cannula 20G	NIISwa Neurovascular Laboratory checklist (MR730.6) Completed imaging request form	Core Physiological Observations Full neurovascular and Neurological observations	Refer to operation report for post procedure requirements' Full Neurological obs 30/60min for 2 hrs Surgical site inspection 30° head up sitting position	Encourage fluid intake post procedure Refer to operation report for post procedure requirements Patients may have CT scan pre and post Myelogram/ Cysternogram Patient weight limits<200Kg (Safe working limit of procedural table)	1hour
Vertebroplasty	Yes	If needed	Minimum 6hrs solids Sip til send clear fluids	Hospital gown IV cannula 20G FBC, U&E, Coags, Group & Hold	NIISwa Neurovascular Laboratory checklist (MR730.6) Completed imaging request form	Core Physiological Observations Full neurovascular and Neurological observations Pain assessment	Refer to operation report for post procedure requirements' Pain assessment Surgical site inspection	Patient weight limits<200Kg (Safe working limit of procedural table)	Variable
Lumbar puncture +/- chemotherapy	Yes	If needed	N/A	Hospital gown FBC, Coags IV cannula 22 or 20G	NIISwa Neurovascular Laboratory checklist (MR730.6) Pathology form Completed imaging request form including name of Dr responsible for cytotoxic drug Administration.	Core Physiological Observations Full Neurovascular and Neurological observations Surgical site inspection	Refer to operation report for post procedure requirements' Surgical site inspection	Haem/Oncology Medical staff administers cytotoxic medication into CSF space. CSF collection by medical team's request. Patient weight limits<200Kg (Safe working limit of procedural table)	1hour

Radiological examination and Neuro interventional procedures

PROCEDURE	WRITTEN CONSENT	CLIP	FASTING	PREPARATION	DOCUMENTATION	OBS PRE	OBS POST	NOTE	Operative time
Lumbar Drain insertion	Yes	If needed	Minimum 6hrs solids Sip til send clear fluids	Hospital gown FBC, Coags IV cannula 20G	NIISwa Neurovascular Laboratory checklist (MR730.6) Completed imaging request form	Core Physiological Observations Full Neurovascular &Neurological observations	Refer to operation report for post procedure requirements' Surgical site inspection	Patient weight limits<200Kg (Safe working limit of procedural table)	1hour
Blood patch	Yes	If needed	4hrs solids Sip til send clear fluids	Hospital gown FBC, Coags IV cannula 20G	NIISwa Neurovascular Laboratory checklist (MR730.6) Completed imaging request form	Core Physiological Observations Full Neurovascular and Neurological observations Pain assessment	Refer to operation report for post procedure requirements' Surgical site inspection Pain assessment	Patient weight limits<200Kg (Safe working limit of procedural table)	1hour
Sclerotherapy	Yes	N/A	Minimum 4hrs solids Sip til send clear fluids	Hospital gown IV cannula 20G FBC	NIISwa Neurovascular Laboratory checklist (MR730.6) Completed imaging request form Pathology form for aspiration sample	Core Physiological Observations Full Neurovascular and Neurological observations	Refer to operation report for post procedure requirements' Pain assessment	May require specimen collection and Fluoro screening Patient weight limits<200Kg (Safe working limit of procedural table)	1hour
Facet joint/Nerve root sleeve injection	Yes	N/A	N/A	Coags, FBC	NIISwa Neurovascular Laboratory checklist (MR730.6) Completed imaging request form	Core Physiological Observations Pain assessment Neurological observations	Refer to operation report for post procedure requirements' Pain assessment Neurological observations	Patient cannot drive for 24 hrs post procedure. Patients taking anti platelet or anticoagulant medication require discussion prior to booking.	Variable

Related Documents

Legislation and Policy:

- Health Services Act 2016
- WA Health Consent to Treatment Policy (MP 0175/22)
- NMHS Clinical Documentation Policy
- NMHS Consent to Treatment Policy
- SCGOPHCG Hospital Policy No 232 Clinical Handover- Communicating for Safety.
- SCGH Hospital Policy Patient Identification & Procedure Matching
- OPH Hospital Policy Patient Identification

Guidelines:

- SCGOPHCG NPG Infusaport- Implantable Intravenous Delivery System
- SCGOPHCG NPG Non-Tunnelled and Tunnelled Large Bore CVC
- SCGOPHCG Clinical Practice Guideline Mupirocin Nasal Ointment for Decolonisation

References

1. Royal Australian & New Zealand College of Radiologists. Standards of Practice for Clinical Radiology Version 11.2, Section 7 Patient Management RANZCR Standards of Practice for Diagnostic and Interventional Radiology
2. Ferral H. The Importance of the Informed Consent for Interventional Radiological Procedures. Journal of Radiology Nursing. 2016 Jan;35(1): 33 - 36. (Level VII)
3. Australian Commission on Safety and Quality in healthcare (ACSQHC) National Safety and Quality Health Service Standards (September 2012). Sydney. ACSQHC, 2021.

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For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

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